



EAM310 • Practice Worksheet

Active Recall • Multiple Choice • True/False • Case Studies
EAM Diagnosis I • Summer 2026 • Jon Centeno • VUIM DAChM

8 Weeks Covered

234 Questions

8 Case Studies

Answer Key in Back

How to use this worksheet

- Work through each week's Section A (recall) → B (MC) → C (T/F) → D (case study), in order.
- Don't peek — the Answer Key is intentionally at the very back, organized the same way.
- For Active Recall, write your answer before checking — recognition isn't the same as recall.
- Case studies build toward a single diagnosis — work the sub-questions before revealing the full pattern.
- S Pen / stylus users: handwrite directly on the blank lines. Adobe Acrobat users: use Fill & Sign → Add Text to type into place — no special form fields needed, it works on any PDF.
- Week 1 now includes Liver/Spleen/Lung/Kidney organ-pattern questions alongside the original Eight Principles material — full CAM Ch.10 coverage. ✨



EAM310 • Worksheet • Eight Principles

FCM Ch.30 • CAM Ch.10 • Active Recall + MC + True/False + Case Study

Week 1 • Jon Centeno • VUIM DAcHM

Name: _____ Date: _____

✨ Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.

What are the Eight Principles?

2.

What two features must occur SIMULTANEOUSLY to indicate an exterior syndrome?

3.

What pulse indicates an exterior syndrome? What pulse indicates interior?

4.

Differentiate Full Heat from Empty Heat — tongue and pulse.

5.

What is the treatment principle for Deficiency? For Excess?

6.

What are the three types of complex Deficiency patterns?

7.

Describe Collapse of Yin vs Collapse of Yang — emergency presentations.

8.

What is 'True Heat with False Cold'?

9.

List the four Qi pathologies and their key features.

10.

What are the three Blood pathologies from CAM Ch.10?

11.

What is the difference between Jin (津) and Ye (液)?

12.

Name the four types of Fluid Retention patterns (internal Chinese terms).

13.

What is 'Sinking of Qi' and how does it differ from Qi Deficiency?

14.

What is Mei He Qi (梅核气) and what causes it?

15.

In the Four Levels system, what level is associated with 'faint skin rashes' and 'dry mouth without strong desire to drink'?

16.

Describe the clinical significance of 'True Cold with False Heat'.

17.

What does it mean that all emotions affect the Heart?

18.

What is the pulse quality classically associated with anger as cause of disease?

19.

Describe the progression: emotions → Qi stagnation → ??

20.

What is Collapse of Yang and how does it differ from Collapse of Yin in clinical presentation?

21.

List the six Liver patterns from CAM Ch.10 and one defining feature of each.

22.

How does Liver-Qi Stagnation differ from Liver-Yang Rising in tongue and pulse?

23.

What distinguishes Spleen-Qi Deficiency from Spleen-Yang Deficiency?

24.

What is 'Sinking of Spleen-Qi' and how does it relate to Spleen-Qi Deficiency?

25.

Differentiate Cold-Damp Encumbering Spleen from Damp-Heat in Spleen/Stomach.

26.

What's the key tongue/pulse difference between Lung-Qi Deficiency and Lung-Yin Deficiency?

27.

How do you tell Phlegm-Damp Obstructing Lung apart from Phlegm-Heat Obstructing Lung?

28.

List the six Kidney patterns from CAM Ch.10.

29.

What's the difference between 'Kidney-Qi Not Firm' and 'Kidney Failing to Receive Qi'?

✨ Section B • Multiple Choice

1.

A patient has sudden chills and fever occurring TOGETHER, headache, and a thin white tongue coat. This is most consistent with:

- | | |
|--|--|
| ☐ (A) Interior Heat pattern | ☐ (B) Exterior pattern (recent invasion) |
| ☐ (C) Empty-Heat from Yin deficiency | ☐ (D) Chronic Spleen-Qi deficiency |

2.

Which combination indicates Empty-Heat rather than Full-Heat?

- ☐ (A) Red tongue, thick yellow coating, forceful rapid pulse
- ☐ (B) Red tongue, no coating, fine rapid pulse
- ☐ (C) Pale tongue, thin white coating, weak slow pulse
- ☐ (D) Purple tongue, sticky coating, wiry pulse

3.

A Wiry pulse is required to confirm which emotion as the cause of disease, per Maciocia?

- ☐ (A) Worry
- ☐ (B) Fear
- ☐ (C) Anger
- ☐ (D) Grief

4.

In the progression Emotions → Qi stagnation → Blood stasis → Heat → Fire → Phlegm, what is typically the EARLIEST visible tongue sign?

- ☐ (A) Purple tongue body
- ☐ (B) Thick yellow coating
- ☐ (C) Red tongue tip
- ☐ (D) Swollen tongue with tooth marks

5.

A patient with hypochondriac pain that moves around, frequent sighing, and a Wiry pulse — but a tongue that's basically normal — most likely has:

- ☐ (A) Liver-Fire Blazing
- ☐ (B) Liver-Qi Stagnation
- ☐ (C) Liver-Yang Rising
- ☐ (D) Liver-Blood Deficiency

6.

Cold limbs, watery clear vaginal discharge, edema, and a pale swollen wet tongue with a Deep-Slow-Weak pulse point to:

- ☐ (A) Spleen-Qi Deficiency
- ☐ (B) Spleen-Yang Deficiency
- ☐ (C) Damp-Heat in Spleen/Stomach
- ☐ (D) Spleen Not Controlling Blood

7.

Breathlessness that is specifically worse on INHALATION, with a weak low voice, points most specifically to:

- ☐ (A) Lung-Qi Deficiency
- ☐ (B) Lung-Yin Deficiency
- ☐ (C) Kidney Failing to Receive Qi
- ☐ (D) Phlegm-Damp Obstructing Lung

8.

Malar flush, night sweats, heat in the '5 centres', dry mouth, and a red tongue with little coating in a Kidney-related case points to:

- ☐ (A) Kidney-Yang Deficiency
- ☐ (B) Kidney-Yin Deficiency
- ☐ (C) Kidney-Qi Not Firm
- ☐ (D) Kidney-Yang Deficiency with Water Overflowing

✨ Section C • True / False

1.

In an Exterior pattern, aversion to cold and fever must occur SIMULTANEOUSLY for the diagnosis to hold.

- ☐ TRUE
- ☐ FALSE

2.

A Full-Heat tongue and an Empty-Heat tongue are both red with a thick yellow coating.

- ☐ TRUE
- ☐ FALSE

3.

Deficiency pain is aggravated by pressure, while Excess pain is relieved by pressure.

- ☐ TRUE
- ☐ FALSE

🐾 Case 1

A 45-year-old woman presents with **afternoon fever, malar flush, heat sensations in the palms and soles, night sweating, insomnia, dry mouth and throat, yellow urine, and dry stools**. Tongue: red, NO coating. Pulse: thready, rapid.

1.

Which Eight Principle pattern(s) does this represent?

2.

Is this Full Heat or Empty Heat? What's the single feature that tells you?

3.

Why does the patient sweat specifically at NIGHT and have heat in the '5 centres' (palms, soles, chest)?



EAM310 • Worksheet • Aetiology

FCM Ch.20–22 • CAM Ch.8 • Active Recall + MC + True/False + Case Study

Week 2 • Jon Centeno • VUIM DAChM

Name: _____ Date: _____

✨ Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.

List the Six Excesses (六淫) and their associated season and organ.

2.

Which pathogenic factor is described as 'chief of the hundred diseases' and why?

3.

What makes Summer-Heat unique among the Six Excesses?

4.

Describe the key features of Dampness that distinguish it from other pathogenic factors.

5.

What is the key clinical feature that differentiates exterior Aversion to Cold from Yang Deficiency cold feeling?

6.

Which emotion is classically associated with each organ? (All 7)

7.

What effect does each of the Seven Emotions have on Qi?

8.

According to Maciocia, when does an emotion become a cause of disease?

9.

What are the five classic signs of overwork's effect on the body per the 'Five Exhaustions'?

10.

How does overwork eventually lead to Yin deficiency?

11.

What tongue changes indicate antibiotic use? Beta-blocker use?

12.

Describe Wind-Heat vs Wind-Cold entry routes and key distinguishing signs.

13.

What is the Shaoyang stage and what are its characteristic symptoms?

14.

Name the three main Liver-related disharmonies arising from anger/emotional disturbance.

15.

What are the 'counteracting emotions' and give two examples.

16.

Describe Joy in its broad TCM sense (not ordinary happiness) and its effect on the Heart.

17.

Describe the three-stage progression of a climatic invasion from Exterior to Interior, and the key sign marking each transition.

18.

What signs result from excessive sexual activity depleting Kidney-Essence?

19.

What classic presentation suggests a parasite-related pattern, especially in a child?

20.

Why does old, unresolved physical trauma matter diagnostically even after the tissue has healed?

✨ Section B • Multiple Choice

1.

A patient has a pale tongue, clear copious urine, and loose stools. Which Eight Principle does this point to?

- | | |
|----------------------------------|------------------------------------|
| ☐ (A) Heat | ☐ (B) Cold |
| ☐ (C) Excess | ☐ (D) Exterior |

2.

Which pulse quality is most associated with the Exterior pattern?

- | | |
|------------------------------------|--------------------------------|
| ☐ (A) Deep | ☐ (B) Wiry |
| ☐ (C) Floating | ☐ (D) Weak |

3.

Pain that is relieved by pressure is characteristic of which pattern?

- ☐ (A) Excess
- ☐ (B) Deficiency
- ☐ (C) Heat
- ☐ (D) Exterior

4.

True Cold with False Heat presents with a flushed face and thirst, but what gives away the TRUE cold underneath?

- ☐ (A) Forceful pulse
- ☐ (B) Patient wants to cover up despite the flush, prefers warm drinks
- ☐ (C) Yellow tongue coating
- ☐ (D) Constipation

5.

Which of the following is the Yin grouping in the Eight Principles?

- ☐ (A) Exterior, Heat, Excess
- ☐ (B) Interior, Cold, Deficiency
- ☐ (C) Exterior, Cold, Excess
- ☐ (D) Interior, Heat, Deficiency

6.

Which stage sits between Exterior and Interior, marked by alternating chills and fever?

- ☐ (A) Taiyang
- ☐ (B) Shaoyang
- ☐ (C) Yangming
- ☐ (D) Jueyin

7.

Excessive sexual activity is understood to primarily deplete which substance?

- ☐ (A) Spleen-Qi
- ☐ (B) Kidney-Essence (Jing)
- ☐ (C) Lung-Yin
- ☐ (D) Heart-Blood

8.

Periumbilical pain that comes and goes, plus teeth-grinding in sleep, is classically associated with:

- ☐ (A) Liver-Qi stagnation
- ☐ (B) Parasites
- ☐ (C) Kidney-Yang deficiency
- ☐ (D) Exterior Wind-Cold

9.

Which emotion can cause disease from sheer intensity alone, without needing to be long-lasting?

- ☐ (A) Worry
- ☐ (B) Pensiveness
- ☐ (C) Shock
- ☐ (D) Sadness

✨ Section C • True / False

1.

Summer-Heat is the only one of the Six Excesses that can occur in any season.

- ☐ TRUE
- ☐ FALSE

2.

Yang-Deficiency cold sensation is relieved by covering with blankets, while exterior Aversion to Cold is not.

- ☐ TRUE
- ☐ FALSE

3.

According to Maciocia, a brief but very intense emotional event can never become a cause of disease — only long-lasting emotions count.

- ☐ TRUE
- ☐ FALSE

 Case 1

A 32-year-old man has **sudden onset of fever and chills occurring together**, headache, stiff neck, runny nose, slight cough. He feels cold and wants to be covered. Tongue: thin white coating. Pulse: floating, tight.

1.

Which Eight Principle pattern is this, and what TWO features confirm it is still Exterior?

2.

Is this Wind-Cold or Wind-Heat? What features led you there?

3.

Why does he specifically want to be covered, and what would it mean if he didn't?



EAM310 • Worksheet • Pulse Diagnosis

Week 3 • Jon Centeno • VUIM DAChM

FCM Ch.25 • Active Recall + MC + True/False + Case Study

Name: _____ Date: _____

✨ Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.

What three qualities define a normal pulse?

2.

Where are the Front, Middle, and Rear positions located, and what Burner does each correspond to?

3.

Floating + Tight pulse with a recent cold — what's the diagnosis?

4.

Floating + Rapid pulse — what's the diagnosis?

5.

What does a Deep + Weak pulse indicate? Deep + Full?

6.

Differentiate Knotted, Hasty, and Intermittent pulses.

7.

What are the four main indications of a Slippery pulse?

8.

What does a Wiry pulse indicate (three conditions)?

9.

What is the difference between a Fine and a Minute pulse?

10.

A pulse that is Hollow — what does it indicate, and what does it feel like?

11.

What is a Moving pulse and what does it indicate?

12.

Give the normal pulse rate range for an adult aged 16-35.

13.

What does the Hammer & Bilton companion mean by a 'pseudo-arrhythmia'?

14.

What are the six ways to classify the 28 pulse qualities?

15.

Why do Small Intestine and Large Intestine have contradictory pulse position assignments across historical texts?

16.

What does a Tight pulse feel like, and what does it indicate?

17.

What is the difference between Overflowing and Full pulses?

18.

A pulse described as 'a Wiry pulse felt only at the deep level' — name it.

19.

What does a Leather pulse feel like and what severe condition does it indicate?

20.

List the procedure steps for taking the pulse, in order, per FCM.

21.

What does it mean for a pulse to have spirit but lack root?

✨ Section B • Multiple Choice

1.

Which pulse quality is described as feeling like a scallion stalk — full at the surface and deep, but empty in the middle?

- ☐ (A) Soggy
- ☐ (C) Leather

- ☐ (B) Hollow
- ☐ (D) Firm

2.

A patient's pulse stops at IRREGULAR intervals and is RAPID. This pulse is called:

- | | |
|-----------------------------------|--|
| ☐ (A) Knotted | ☐ (B) Intermittent |
| ☐ (C) Hasty | ☐ (D) Hurried |

3.

Which TWO pulse qualities, when found together, most classically indicate Wind-Heat invasion?

- | | |
|--|--|
| ☐ (A) Deep + Slow | ☐ (B) Floating + Tight |
| ☐ (C) Floating + Rapid | ☐ (D) Wiry + Slippery |

4.

On the LEFT wrist, which organ is felt at the Middle (guan) position?

- | | |
|----------------------------------|----------------------------------|
| ☐ (A) Heart | ☐ (B) Liver |
| ☐ (C) Kidney | ☐ (D) Spleen |

5.

A Slippery pulse found in an otherwise healthy young woman of reproductive age most likely indicates:

- | | |
|--|--|
| ☐ (A) Phlegm accumulation | ☐ (B) Pregnancy |
| ☐ (C) Severe Qi deficiency | ☐ (D) Blood stasis |

6.

Per the Hammer & Bilton companion, what should you do FIRST when you feel an irregular pulse beat?

- | | |
|--|--|
| ☐ (A) Immediately diagnose a Knotted pulse | ☐ (B) Re-check with the patient still and breathing calmly |
| ☐ (C) Switch to feeling the apical pulse instead | ☐ (D) Conclude it is a serious heart condition |

✨ Section C • True / False

1.

A Knotted pulse is Rapid with irregular stops, while a Hasty pulse is Slow with irregular stops.

- ☐ TRUE ☐ FALSE

2.

The Intermittent pulse, which stops at REGULAR intervals, is considered more serious than Knotted or Hasty.

- ☐ TRUE ☐ FALSE

3.

A pseudo-arrhythmia should be diagnosed as a true Knotted/Hasty/Intermittent pulse without re-checking the patient.

- ☐ TRUE ☐ FALSE

 Case 1

A patient reports their pulse feels **'rolling and smooth, like marbles under the skin.'** They have a sticky taste in their mouth, feel heavy and foggy, and have a swollen tongue with a greasy white coating.

1.

Name the pulse quality being described.

2.

What does this pulse quality indicate here, and what confirms it (rather than, say, pregnancy)?

3.

Why does the patient feel 'heavy and foggy' specifically?

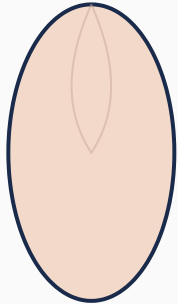


EAM310 • Worksheet • Tongue Diagnosis

Week 4 • Jon Centeno • VUIM DAcHM

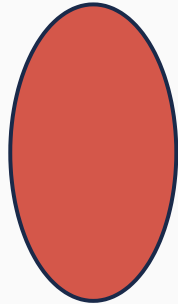
FCM Ch.23 • Active Recall + MC + True/False + Case Study

Name: _____ Date: _____



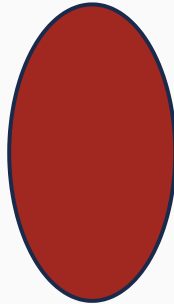
Pale

Yang/Blood
deficiency



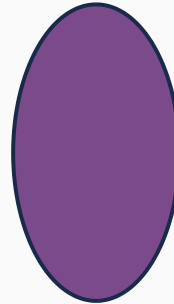
Red

Heat (Full if
coated, Empty if not)



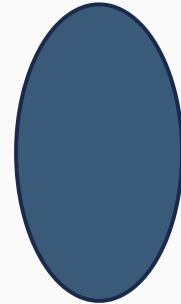
Deep-Red

Same as Red,
more severe



Purple

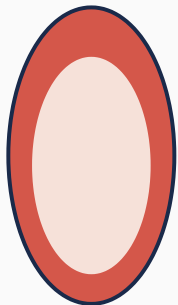
Always Blood
stasis



Blue

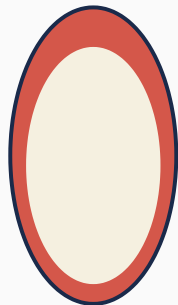
Interior Cold +
Blood stasis

Original diagram • 5 Pathological Body Colors • stylized reference, not diagnostic-grade color matching



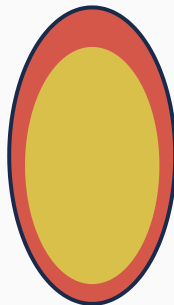
Thin White

Normal, or
early exterior



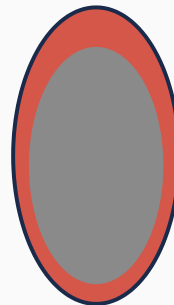
Thick White

Excess Cold or
Dampness



Yellow

Heat (deeper
= more severe)



Grey

Interior Heat or
Cold-Damp



Black

Extreme Heat
or extreme Cold

Original diagram • Tongue Coating Colors • body color shown as base Red for contrast only

✨ Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.

A Red tongue with a thick coating vs. a Red tongue with no coating — what's the difference?

2.

What does a Purple tongue ALWAYS indicate, per Maciocia?

3.

Differentiate the 'Heart crack' from the 'Stomach crack.'

4.

A Pale tongue that is wet vs. a Pale tongue that is dry — what's the difference?

5.

What does a Swollen tongue with a swollen RED tip indicate? Swollen red SIDES?

6.

What does Tooth-marking on the tongue indicate?

7.

What does a Deviated tongue (deviating to one side on protrusion) indicate?

8.

Map the tongue microsystem: which organ corresponds to tip, center, root, and sides?

9.

What does a Stiff tongue indicate? A Flaccid tongue?

10.

Which Maciocia Tongue Companion chapters map to Week 4's content?

11.

What does a Long tongue indicate? A Short tongue?

12.

What does a Flaccid tongue indicate?

13.

Differentiate a swollen RED tip from wide swollen PALE sides on the tongue.

14.

What does a swelling specifically in the front third of the tongue indicate?

15.

Orangey/dark pale sides on the tongue indicate what severity of Liver-Blood deficiency?

16.

What is the relationship between tongue body color and Blood/Nutritive Qi vs tongue coating and the Yang organs?

✨ Section B • Multiple Choice

1.

A tongue that is Red with NO coating at all (peeled, mirror-smooth) most strongly indicates:

- | | |
|--|--|
| ☐ (A) Full-Heat from an acute invasion | ☐ (B) Empty-Heat from Yin deficiency |
| ☐ (C) Interior Cold | ☐ (D) Phlegm-Dampness |

2.

A 'Stomach crack' on the tongue is best described as:

- ☐ A A long deep crack reaching the tip
- ☐ B A shallow wide crack NOT reaching the tip
- ☐ C A horizontal crack on the sides only
- ☐ D Multiple small ice-floe-like cracks

3.

Tooth-marking on the tongue edges is classically attributed to:

- ☐ A Liver-Blood deficiency
- ☐ B Spleen-Qi deficiency
- ☐ C Kidney-Yin deficiency
- ☐ D Heart-Fire

4.

Per Maciocia, a Purple tongue body color ALWAYS indicates which underlying process?

- ☐ A Qi stagnation
- ☐ B Blood stasis
- ☐ C Phlegm accumulation
- ☐ D Yin deficiency

5.

Which tongue zone corresponds to the Kidney in the standard tongue microsystem map?

- ☐ A Tip
- ☐ B Center
- ☐ C Root
- ☐ D Sides

✨ Section C • True / False

1.

A Purple tongue, per Maciocia, always indicates Qi stagnation rather than Blood stasis.

- ☐ TRUE
- ☐ FALSE

2.

Tooth-marking on the tongue edges indicates Spleen-Qi deficiency.

- ☐ TRUE
- ☐ FALSE

3.

A coating that looks 'added on' rather than rooted in the tongue body indicates severe Stomach-Yin and Qi exhaustion.

- ☐ TRUE
- ☐ FALSE

🐾 Case 1

A tongue is **purple on the sides only**, in a female patient with **painful periods and dark, clotted menstrual blood**.

1.

What does Purple on the tongue ALWAYS indicate, per Maciocia?

2.

Why specifically the SIDES, and what does that localize to?

3.

What does the dark, clotted character of the menstrual blood add to the picture?



✨ Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.

What are the four observable signs of 'having Spirit' (Shen)?

2.

List the four attributes of a normal complexion.

3.

A patient's whole face is red vs. only their cheekbones are red — what's the difference?

4.

What does a green complexion with red eyes indicate? Grass-like green?

5.

Map the eye zones: which organ corresponds to corners, upper lid, lower lid, sclera, iris, pupil?

6.

What does the tip of the nose reflect? The bridge of the nose?

7.

A tongue coating that looks 'added on' rather than growing from the tongue — what does this indicate?

8.

Differentiate a partially peeled tongue from a completely peeled (glossy) tongue.

9.

What does a 'floating' complexion color indicate vs. a 'deep' complexion color?

10.

What does a black complexion that is moist indicate vs. dry/burned-looking?

11.

What does a 'distinct' complexion color indicate vs an 'obscure' one?

12.

What does a thick coating on the tongue without root indicate, compared to a thin coating without root?

13.

Whole eye red, painful, and swollen — what two patterns could this represent?

14.

What does swelling under the eyes indicate?

15.

Describe the difference between Damp-Heat with Heat predominant vs Dampness predominant, as shown on the complexion.

✨ Section B • Multiple Choice

1.

A patient's complexion is bright white with a bluish tinge. This most likely indicates:

- | | |
|--|--|
| ☐ (A) Blood deficiency | ☐ (B) Yang deficiency with pronounced Cold |
| ☐ (C) Damp-Heat | ☐ (D) Liver-Fire |

2.

Which of these is NOT one of the four observable signs of 'having Spirit' (Shen)?

- | | |
|---|--|
| ☐ (A) Lustre of the complexion | ☐ (B) A strong, forceful pulse |
| ☐ (C) Clarity and alertness of mind | ☐ (D) Even breathing |

3.

The tip of the nose corresponds to which organ in facial zone mapping?

- | | |
|----------------------------------|----------------------------------|
| ☐ (A) Liver | ☐ (B) Kidney |
| ☐ (C) Spleen | ☐ (D) Heart |

4.

A tongue coating that appears 'glued on' rather than emerging naturally from the tongue surface is described as lacking:

- | | |
|------------------------------------|-------------------------------------|
| ☐ (A) Moisture | ☐ (B) Thickness |
| ☐ (C) Root | ☐ (D) Color |

✨ Section C • True / False

1.

A patient who 'has Spirit' (Shen) shows a lustrous complexion, vital eyes, a clear mind, and even breathing.

- ☐ TRUE ☐ FALSE

2.

Cheekbone-only facial redness indicates Full-Heat, while redness across the whole face indicates Empty-Heat.

- ☐ TRUE ☐ FALSE

3.

On the eye zone map, the pupil corresponds to the Kidney and the iris corresponds to the Liver.

- ☐ TRUE ☐ FALSE

 Case 1

A patient's complexion is **sallow yellow**, with a tendency to fatigue, poor appetite, and a tongue that is **pale with tooth marks**.

1.

What does a sallow yellow complexion most commonly indicate?

2.

How does the tooth-marked, pale tongue support this conclusion?

3.

Why does Spleen deficiency typically cause both fatigue AND poor appetite together, rather than just one?



EAM310 • Worksheet • Inquiry — 16

Questions

FCM Ch.24 • Active Recall + MC + True/False + Case Study

Week 7 • Jon Centeno • VUIM DACHM

Name: _____ Date: _____

Reference — The Classical 10 Questions (十问歌)

1. Cold and heat 2. Sweat 3. Head and body 4. Stool and urine 5. Food and drink 6. Chest and abdomen 7. Deafness/hearing 8. Thirst 9. Old disease 10. Cause. Maciocia's 16 expand and reorder these — same tradition, modernized for clinical practice. The full 16 with example question phrasing ("How's your energy through the day?" etc.) is in the Workbook's Inquiry Reference page and the StudyApp Week 7 tab.

Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.
Why did Maciocia move 'Pain' to question #1 in his revised 16 questions, instead of 'chills and fever' (the classical first question)?

2.
What are the three NEW questions Maciocia added to the classical 10?

3.
Desire for large amounts of cold water vs. thirst with small sips — what's the difference?

4.
Thirst but NO desire to actually drink — what does this indicate?

5.
Daytime sweating vs. night-time sweating — what's the difference?

6.

What does 'five-palm sweat' indicate?

7.

Pain before urination vs. during vs. after — differentiate.

8.

Borborygmi with loose stools vs. borborygmi with distension and NO loose stools — differentiate.

9.

What is the significance of profuse, oily forehead sweat that doesn't flow, 'like pearls'?

10.

What is the difference between sweating only on the hands versus the whole body?

11.

Frequent + copious urine vs frequent + scanty urine — differentiate.

12.

What does scanty, dark urine indicate?

13.

What is the order of Maciocia's procedure for a full intake, from first contact to determining cause?

14.

Black or very dark stools indicate what pattern?

✨ Section B • Multiple Choice

1.

A patient reports thirst but states they have no real desire to actually drink water when offered. This combination most specifically points to:

- | | |
|-------------------------------------|---|
| ☐ (A) Full-Heat | ☐ (B) Yin deficiency |
| ☐ (C) Damp-Heat | ☐ (D) Yang deficiency |

2.

Why did Maciocia move 'chills and fever' from question #1 (classical order) to question #12 in his revised 16 questions?

- | | |
|--|---|
| ☐ (A) It is no longer clinically relevant | ☐ (B) Western patients rarely present with acute febrile disease as their chief complaint |
| ☐ (C) It duplicates information from pulse diagnosis | ☐ (D) Patients find it an uncomfortable question to answer early |

3.

Borborygmi (intestinal gurgling) WITH loose stools points toward:

- | | |
|--|---|
| ☐ (A) Liver-Qi stagnation | ☐ (B) Spleen deficiency |
| ☐ (C) Kidney-Yang deficiency | ☐ (D) Blood stasis |

4.

Pain that occurs specifically DURING urination points to:

- | | |
|---|--|
| ☐ (A) Qi stagnation in the Lower Burner | ☐ (B) Damp-Heat in the Bladder |
| ☐ (C) Kidney-Yang deficiency | ☐ (D) Qi deficiency |

✨ Section C • True / False

1.

Maciocia moved Pain to question #1 in his 16-question system because chills/fever is rarely a Western patient's chief complaint.

☐ TRUE ☐ FALSE

2.

Thirst with a desire for only small sips of water typically indicates Full-Heat.

☐ TRUE ☐ FALSE

3.

Pain that occurs BEFORE urination indicates Qi deficiency.

☐ TRUE ☐ FALSE

 Case 1

A patient reports feeling **thirsty but has no real desire to drink water** — when they do drink, they only take small amounts. They also have a **sticky taste in their mouth, heaviness, and epigastric fullness**.

1.

What does 'thirst without desire to drink' point toward, and why?

2.

How does the sticky taste + epigastric fullness support Damp-Heat specifically (vs simple Heat alone)?

3.

If this patient instead desired LARGE amounts of cold water, what would that change?



EAM310 • Worksheet • Inquiry + Palpation

Week 8 • Jon Centeno • VUIM DAChM

FCM Ch.24–25 • Active Recall + MC + True/False + Case Study

Name: _____ Date: _____

✨ Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.

What is the single general rule distinguishing Empty pain from Full pain?

2.

Differentiate pain from Qi stagnation vs. pain from Blood stasis.

3.

A headache at the vertex — which channel, and most likely cause?

4.

A headache that is distending and throbbing — what's the likely cause?

5.

Epigastric pain that is dull and alleviated by eating vs. severe and aggravated by eating — differentiate.

6.

Numbness of fingers/elbow/arm on ONE side only — what is the clinical concern?

7.

On abdominal palpation, what does a mass that MOVES under the fingers indicate vs. a FIXED hard mass?

8.

What does tenderness at Ren-17 (Shanzhong) on light palpation indicate?

9.

What does it mean if chest palpation RELIEVES discomfort?

10.

What does pain from Cold feel like, and what aggravates/alleviates it?

11.

What does pain from Damp-Heat feel like?

12.

Differentiate a headache that's worse in the daytime vs the evening vs at night.

13.

What does pain at the hypogastrium (lowest abdominal area) typically indicate?

14.

What does the area below the xiphoid process ('Xu Li') reflect, and what does a hard/knotted finding there mean?

✨ Section B • Multiple Choice

1.
Pain that comes and goes with emotional state, causing more distension than actual pain, with no fixed location, is most characteristic of:

☐ (A) Blood stasis	☐ (B) Qi stagnation
☐ (C) Damp-Heat	☐ (D) Cold
2.
On chest palpation, tenderness elicited even with LIGHT pressure at Ren-17 (Shanzhong) most specifically suggests:

☐ (A) Lung-Qi deficiency	☐ (B) Heart-Blood stasis
☐ (C) Stomach-Heat	☐ (D) Kidney-Yang deficiency
3.
Numbness localized to the fingers/arm on ONE side only is a clinical warning sign for:

☐ (A) Simple Blood deficiency	☐ (B) Possible impending Wind-stroke
☐ (C) Spleen-Qi deficiency	☐ (D) Normal aging
4.
An abdominal mass that is FIXED, hard, and does not move under palpation most likely represents:

☐ (A) Qi stagnation	☐ (B) Blood stasis
☐ (C) Simple bloating	☐ (D) Phlegm-Dampness

✨ Section C • True / False

1.
An abdominal mass that MOVES under palpation suggests Blood stasis, while a FIXED hard mass suggests Qi stagnation.
☐ TRUE ☐ FALSE
2.
Tenderness at Ren-17 elicited even with light pressure is a classic sign of Heart-Blood stasis.
☐ TRUE ☐ FALSE
3.
Unilateral numbness of the fingers and arm should be dismissed as a minor, clinically insignificant finding.
☐ TRUE ☐ FALSE

 Case 1

A patient has **epigastric pain that is severe, aggravated by eating, and worse with pressure**. They also report **foul breath and constipation**. Tongue: thick yellow coating.

1.

Is this Full or Empty type pain? What's the single biggest clue?

2.

What is the likely underlying cause, given foul breath + constipation + thick yellow coat?

3.

Why does eating specifically worsen this patient's pain?



EAM310 • Worksheet • Palpation +

Listening/Smelling

FCM Ch.25-26 • Active Recall + MC + True/False + Case Study

Week 9 • Jon Centeno • VUIM DACHM

Name: _____ Date: _____

✨ Section A • Active Recall

Cover the workbook, answer from memory, then check yourself against the Answer Key at the back.

1.

What is the single general rule for diagnosis by listening (hearing)?

2.

What is the single general rule for diagnosis by smelling?

3.

Sudden loss of voice vs. gradual loss of voice — what's the difference?

4.

Match the Five-Element voice types to their organs: shouting, laughing, singing, weeping, groaning.

5.

What does a dry cough specifically indicate?

6.

Why is it diagnostically meaningful if a Wood-type person's body odor is scorched rather than rancid?

7.

What is a 'pseudo-arrhythmia' and how do you clinically rule it out?

8.

What does it mean if the apical (heart apex) pulse and the radial pulse disagree?

9.

Intestinal gas with no smell at all — what does this indicate?

10.

What does it mean if breath odor is foul AND pungent, versus simply sour?

11.

What does it mean if vomiting occurs with a loud noise vs a low quiet sound?

12.

What does a feeble apical (heart apex) beat without strength indicate?

13.

What does cold skin specifically over the lower back, loins, and feet indicate?

14.

Describe how a constitutional vs pathological body odor mismatch is diagnostically meaningful, using the Wood-type example.

✨ Section B • Multiple Choice

1.

A patient speaks very little and seems reluctant to talk. This voice pattern most often indicates:

- ☐ (A) Heat pattern
- ☐ (B) Cold pattern or Lung-Qi deficiency
- ☐ (C) Liver-Fire
- ☐ (D) Stomach-Heat

2.

Per the Five-Element voice correspondences, a 'groaning' voice quality points to which organ?

- ☐ (A) Liver
- ☐ (B) Heart
- ☐ (C) Spleen
- ☐ (D) Kidney

3.

Per Hammer & Bilton, what is a 'pseudo-arrhythmia'?

- ☐ (A) A serious cardiac arrhythmia requiring referral
- ☐ (B) An irregularity caused by a benign, transient factor rather than true pathology
- ☐ (C) A pulse quality unique to elderly patients
- ☐ (D) A type of Knotted pulse seen only in Cold patterns

4.

A discrepancy between the apical (heart apex) pulse and the radial wrist pulse generally indicates:

- ☐ (A) A normal finding requiring no further attention
- ☐ (B) Good prognosis
- ☐ (C) Poor prognosis
- ☐ (D) Pregnancy

✨ Section C • True / False

1.

A loud, forceful sound on auscultation generally indicates a Full pattern, while a weak sound indicates an Empty pattern.

- ☐ TRUE ☐ FALSE

2.

An absent or faint body odor generally points toward Heat, while a strong foul odor points toward Cold.

- ☐ TRUE ☐ FALSE

3.

A discrepancy between the apical (heart apex) pulse and the radial wrist pulse generally indicates a good prognosis.

- ☐ TRUE ☐ FALSE

 Case 1

A patient speaks in a **loud, shouting-toned voice**, is irritable, and has a body odor that is **faintly rancid**. They report chronic frustration and a **Wiry pulse**.

1.

Which organ does this voice/odor pattern correspond to in Five-Element correspondence?

2.

Does the LOUD quality of the voice tell you Full or Empty? Why?

3.

How does the Wiry pulse complete (not just add to) this picture?



Answer Key

No peeking until you've actually tried. We mean it. 🐾



EAM310 • Worksheet Answer Key

Eight Principles & Aetiology

Jon Centeno • VUIM DAChM

Answer Key – cover this page while testing yourself

✨ Eight Principles

Section A • Active Recall

1. What are the Eight Principles?

→ Four pairs of opposites: Exterior/Interior, Cold/Heat, Deficiency/Excess, and Yin/Yang. Yin and Yang are the master pair that summarize the other six.

2. What two features must occur SIMULTANEOUSLY to indicate an exterior syndrome?

→ Aversion to cold (subjective) AND fever/body feeling hot on palpation (objective) occurring at the same time. The moment aversion to cold disappears, the pathogen has moved interior.

3. What pulse indicates an exterior syndrome? What pulse indicates interior?

→ Exterior = Floating pulse (浮脉). Interior = Deep pulse (沉脉).

4. Differentiate Full Heat from Empty Heat — tongue and pulse.

→ Full Heat: Red tongue with THICK YELLOW coating; pulse Rapid AND FORCEFUL. Empty Heat (Yin def): Red tongue with LITTLE OR NO COATING (peeled); pulse Rapid AND FINE/THREADY.

5. What is the treatment principle for Deficiency? For Excess?

→ Deficiency → Tonify (补 bǔ). Excess → Reduce/Expel (泻 xiè). Never tonify in an exterior condition — it pushes the pathogen inward.

6. What are the three types of complex Deficiency patterns?

→ (1) Deficiency complicated by Excess — pathogen + insufficient zheng qi. (2) Transformation of excess to deficiency — long-term excess consuming zheng qi. (3) Deficiency with false excess manifestations.

7. Describe Collapse of Yin vs Collapse of Yang — emergency presentations.

→ Collapse of Yin: sticky sweat, feverish, warm hands/feet, irritability, thirst (cold drinks), red dry tongue, thready rapid weak pulse. Collapse of Yang: cold sweat like pearls, cold body/limbs, listlessness, no thirst (or prefers hot), pale moist tongue, thready fading pulse.

8. What is 'True Heat with False Cold'?

→ Interior heat so extreme that it blocks Yang from reaching the extremities → cold limbs despite raging interior heat. KEY: pulse is deep and FORCEFUL — this reveals the true nature. Also: constipation, thirst, scanty dark urine.

9. List the four Qi pathologies and their key features.

→ Qi Deficiency: fatigue, spontaneous sweating, pale tongue, deficiency pulse. Qi Sinking: above + bearing-down sensation, prolapse. Qi Stagnation: distension > pain, waxes/wanes, wiry pulse. Rebellious Qi: cough/asthma (Lung), nausea/vomiting (Stomach), headache/hemoptysis (Liver).

10. What are the three Blood pathologies from CAM Ch.10?

→ Blood Deficiency (pallor, dizziness, thready pulse), Blood Stasis (fixed stabbing pain, mass, purple tongue, hesitant pulse), Heat in Blood (restlessness, bleeding, deep red tongue, rapid pulse).

11. What is the difference between Jin (津) and Ye (液)?

→ Jin: clear and thin, fills vessels, moistens muscles and skin, easily consumed and replenished (Yang-type). Ye: thick and heavy, stored in joints and orifices, nourishes brain, marrow, joints — harder to replace (Yin-type).

12. Name the four types of Fluid Retention patterns (internal Chinese terms).

→ Tanyin (痰饮) — stomach/intestines; Xuanyin (悬饮) — hypochondrium; Yiyin (溢饮) — body surface/anasarca; Zhiyin (支饮) — chest/epigastrium.

13. What is 'Sinking of Qi' and how does it differ from Qi Deficiency?

→ Sinking of Qi is a subtype of Qi Deficiency — always has all Qi Def signs PLUS a bearing-down sensation in the abdomen and organ prolapse (anus, uterus, stomach, kidney). Treat by raising and tonifying middle Qi. Cannot have Sinking without Deficiency.

14. What is Mei He Qi (梅核气) and what causes it?

→ Plum-pit qi — sensation of something stuck in the throat that can't be swallowed or expectorated. Nothing found on endoscopy. Caused by Liver Qi stagnation + Phlegm accumulation. Key formula: Ban Xia Hou Po Tang.

15. In the Four Levels system, what level is associated with 'faint skin rashes' and 'dry mouth without strong desire to drink'?

→ Ying (Nutrient) level — febrile pathogen has injured ying yin and is disturbing the Heart/Mind. Deep red tongue, thready rapid pulse. Treatment: clear heat from ying, cool blood.

16. Describe the clinical significance of 'True Cold with False Heat'.

→ Extreme interior cold forces Yang to the surface → flushed face despite interior cold. Patient wants to COVER UP despite appearing hot. Takes warm drinks to relieve apparent thirst. Pulse is superficial but WEAK. Pale tongue. Treat the underlying cold.

17. What does it mean that all emotions affect the Heart?

→ The Heart houses the Mind (Shen) — it is the organ responsible for consciousness and recognizing emotions. So while each emotion targets a specific organ (anger → Liver etc.), all emotional disturbance is ultimately registered and felt by the Heart. Red tongue TIP = common sign of emotional stress regardless of which organ is primarily affected.

18. What is the pulse quality classically associated with anger as cause of disease?

→ Wiry pulse (弦脉). Maciocia: 'When anger is the cause of disease, the pulse is Wiry: if the pulse is not Wiry, then anger is not the cause of the disease.'

19. Describe the progression: emotions → Qi stagnation → ??

→ Emotions → Qi stagnation → Blood stasis (especially in women, affects Heart/Liver/Uterus) → Heat → Fire (five emotions can turn into Fire) → Phlegm (Qi disruption impairs fluid metabolism). Red tongue tip is the earliest sign this process is occurring.

20. What is Collapse of Yang and how does it differ from Collapse of Yin in clinical presentation?

→ Collapse of Yang = extreme Yang Qi exhaustion → cold sweat like pearls, cold body/limbs, feeble breathing, listlessness, no thirst (or prefers hot), pale moist tongue, thready fading pulse. Collapse of Yin = massive Yin consumption → sticky warm sweat, feverish warm hands/feet, restlessness, thirst (cold drinks), red dry tongue, thready rapid weak pulse.

21. List the six Liver patterns from CAM Ch.10 and one defining feature of each.

→ Liver-Qi Stagnation (distension > pain, Wiry pulse), Liver-Yang Rising (dizziness/headache + lower-body deficiency signs), Liver-Fire Blazing (severe headache, red eyes, yellow coat, forceful Wiry-Rapid pulse), Liver-Blood Deficiency (pale, blurred vision, Choppy pulse), Liver-Wind (tremor/convulsion, possible Wind-stroke), Cold Stagnating in Liver Channel (genital/lower abdominal pain, Wiry-Tight pulse).

22. How does Liver-Qi Stagnation differ from Liver-Yang Rising in tongue and pulse?

→ Liver-Qi Stagnation: tongue normal or slightly purple sides, pulse simply Wiry. Liver-Yang Rising: tongue red (esp. sides), pulse Wiry possibly Wiry-Thready (because it's rooted in underlying Yin deficiency, not pure Excess).

23. What distinguishes Spleen-Qi Deficiency from Spleen-Yang Deficiency?

→ Spleen-Yang deficiency = Spleen-Qi deficiency PLUS cold signs: cold limbs, cold abdominal pain relieved by warmth, watery discharge, edema. Tongue becomes more swollen/wet; pulse becomes Deep-Slow-Weak instead of just Weak/Empty.

24. What is 'Sinking of Spleen-Qi' and how does it relate to Spleen-Qi Deficiency?

→ It's a subtype/complication of Spleen-Qi deficiency where Middle Qi fails to hold organs in place — bearing-down sensation, chronic diarrhea, and prolapse (stomach, uterus, rectum, bladder) on top of ordinary Qi-deficiency signs.

25. Differentiate Cold-Damp Encumbering Spleen from Damp-Heat in Spleen/Stomach.

→ Cold-Damp: white sticky/greasy coat, Soggy or Slow pulse, no thirst, foggy head. Damp-Heat: yellow greasy coat, Soggy-Rapid or Slippery-Rapid pulse, thirst with no desire to drink, possible jaundice and dark urine.

26. What's the key tongue/pulse difference between Lung-Qi Deficiency and Lung-Yin Deficiency?

→ Lung-Qi def: pale tongue, Empty/Weak pulse (esp. Right Front) — weak voice, SOB on exertion, spontaneous sweating. Lung-Yin def: red tongue with little coating (esp. front third), Floating-Empty or Thready-Rapid pulse — dry cough, malar flush, night sweats.

27. How do you tell Phlegm-Damp Obstructing Lung apart from Phlegm-Heat Obstructing Lung?

→ Phlegm-Damp: white sticky coat, Slippery pulse, copious white sputum. Phlegm-Heat: red tongue with yellow sticky coat, Slippery-Rapid pulse, thick yellow (possibly blood-streaked) sputum, fever, thirst.

28. List the six Kidney patterns from CAM Ch.10.

→ Kidney-Yang Deficiency, Kidney-Yin Deficiency, Kidney-Essence Deficiency, Kidney-Qi Not Firm, Kidney Failing to Receive Qi, and Kidney-Yang Deficiency with Water Overflowing.

29. What's the difference between 'Kidney-Qi Not Firm' and 'Kidney Failing to Receive Qi'?

→ Kidney-Qi Not Firm = consolidation failure: frequent/incontinent urination, enuresis, premature ejaculation, leaking discharge, uterine prolapse. Kidney Failing to Receive Qi = the Kidney's role anchoring inhaled Qi fails: chronic asthma/dyspnea worse on exertion, breathlessness worse on INHALATION specifically.

Section B • Multiple Choice

1. A patient has sudden chills and fever occurring TOGETHER, headache, and a thin white tongue coat. This is most consistent with:

→ B. Exterior pattern (recent invasion) — Simultaneous chills and fever at disease onset is the hallmark of an Exterior pattern — the body fighting an invading pathogen at the surface.

2. Which combination indicates Empty-Heat rather than Full-Heat?

→ B. Red tongue, no coating, fine rapid pulse — Empty-Heat (Yin deficiency) shows a red tongue with NO coating (peeled) and a fine/thready rapid pulse — the lack of coating signals deficiency, not an active pathogen.

3. A Wiry pulse is required to confirm which emotion as the cause of disease, per Maciocia?

→ C. Anger — Maciocia states explicitly: when anger is the cause of disease, the pulse must be Wiry — if not Wiry, anger is not the cause.

4. In the progression Emotions → Qi stagnation → Blood stasis → Heat → Fire → Phlegm, what is typically the EARLIEST visible tongue sign?

→ C. Red tongue tip — A red tongue tip is the earliest visible sign of emotional disturbance beginning to generate Heat, since the Heart (reflected at the tip) registers all emotional stress first.

5. A patient with hypochondriac pain that moves around, frequent sighing, and a Wiry pulse — but a tongue that's basically normal — most likely has:

→ B. Liver-Qi Stagnation — Liver-Qi Stagnation is the 'purest' Qi-level pattern: moving distension/pain, Wiry pulse, and often a near-normal tongue, since Blood and Yin haven't yet been affected.

6. Cold limbs, watery clear vaginal discharge, edema, and a pale swollen wet tongue with a Deep-Slow-Weak pulse point to:

→ B. Spleen-Yang Deficiency — The added Cold signs (cold limbs, watery discharge, edema) plus the Deep-Slow-Weak pulse mark this as Spleen-YANG deficiency, a step beyond simple Spleen-Qi deficiency.

7. Breathlessness that is specifically worse on INHALATION, with a weak low voice, points most specifically to:

→ C. Kidney Failing to Receive Qi — The Kidney's job is to 'grasp' or receive Qi sent down by the Lung on inhalation — when this fails, breathlessness is characteristically worse breathing IN, not out.

8. Malar flush, night sweats, heat in the '5 centres', dry mouth, and a red tongue with little coating in a Kidney-related case points to:

→ B. Kidney-Yin Deficiency — Heat-from-deficiency signs (malar flush, night sweats, 5-centre heat, dry mouth, red tongue no coat) are the signature of Kidney-YIN deficiency, the mirror image of the cold/wet Kidney-Yang deficiency presentation.

Section C • True/False

1. In an Exterior pattern, aversion to cold and fever must occur SIMULTANEOUSLY for the diagnosis to hold.

→ TRUE — True. The hallmark of Exterior syndrome is BOTH occurring together; once aversion to cold resolves on its own, the pathogen has already moved Interior.

2. A Full-Heat tongue and an Empty-Heat tongue are both red with a thick yellow coating.

→ FALSE — False. Full-Heat = red tongue WITH thick yellow coating. Empty-Heat = red tongue with NO coating (peeled) — coating presence is the differentiator.

3. Deficiency pain is aggravated by pressure, while Excess pain is relieved by pressure.

→ FALSE — False — it's the reverse. Deficiency pain is relieved (likes) pressure; Excess pain is aggravated (dislikes) pressure.

Section D • Case Study

1. Which Eight Principle pattern(s) does this represent?

→ Interior · Heat · Deficiency. (Also Yin in the master Yin/Yang pairing.)

2. Is this Full Heat or Empty Heat? What's the single feature that tells you?

→ Empty Heat. The tongue has NO coating at all (peeled) — Full Heat would show a THICK YELLOW coating instead. The thready (not forceful) pulse confirms deficiency underneath the rapid rate.

3. Why does the patient sweat specifically at NIGHT and have heat in the '5 centres' (palms, soles, chest)?

→ Classic Yin-deficiency Empty-Heat pattern: insufficient Yin fails to anchor Yang at night, so Yang/Heat floats and escapes as night sweat; the 5-centre heat reflects deficient Yin failing to cool the periphery.

Pattern Reveal

★. Full pattern + reasoning

→ Empty Heat from Yin Deficiency. Reasoning: gradual onset, no aversion to cold (Interior), heat signs without coating (Empty not Full), thready pulse (deficiency under the rapid rate). Treatment principle: nourish Yin to anchor and control the Heat — never simply 'clear heat' with cold/cool herbs the way you would for Full Heat, or you'll further damage Yin.

✨ Aetiology

Section A • Active Recall

1. List the Six Excesses (六淫) and their associated season and organ.

→ Wind → Spring → Liver; Cold → Winter → Kidneys; Summer-Heat → Summer → Heart (summer only!); Dampness → Late Summer → Spleen; Dryness → Autumn → Lungs; Fire/Heat → Summer (also endogenous) → Heart.

2. Which pathogenic factor is described as 'chief of the hundred diseases' and why?

→ Wind — because it is mobile and easily combines with other pathogenic factors (Wind-Cold, Wind-Heat, Wind-Damp), and it tends to open the pores, allowing other pathogens to invade.

3. What makes Summer-Heat unique among the Six Excesses?

→ It can ONLY occur in summer — it is the only climatic factor that cannot arise outside its associated season. All others (Wind, Cold, Damp, Dryness, Fire) can occur in any season depending on circumstances.

4. Describe the key features of Dampness that distinguish it from other pathogenic factors.

→ Gradual invasion, heaviness of limbs/body, turbid/sticky discharges, fullness in chest and epigastrium, tendency toward prolonged and intractable disease, greasy tongue coat, soggy pulse. Dampness is heavy and turbid — like mud vs wind.

5. What is the key clinical feature that differentiates exterior Aversion to Cold from Yang Deficiency cold feeling?

→ Exterior Aversion to Cold: sudden onset, NOT relieved by covering, occurs simultaneously with fever. Yang Deficiency cold: chronic, IS relieved by covering with blankets, no simultaneous fever.

6. Which emotion is classically associated with each organ? (All 7)

→ Anger → Liver; Joy (overstimulation) → Heart; Sadness/Grief → Lungs & Heart; Worry → Lungs & Spleen; Pensiveness → Spleen; Fear → Kidneys; Shock → Heart.

7. What effect does each of the Seven Emotions have on Qi?

→ Anger = Qi rises; Joy = Qi slows; Sadness = Qi dissolves; Worry = Qi knots; Pensiveness = Qi knots; Fear = Qi descends; Shock = Qi scatters.

8. According to Maciocia, when does an emotion become a cause of disease?

→ When it is (1) long-lasting (months or years) OR (2) very intense (even if brief). Shock is the exception — can become a cause almost instantly due to its intensity. Normal emotions in normal circumstances do NOT cause disease.

9. What are the five classic signs of overwork's effect on the body per the 'Five Exhaustions'?

→ Excessive use of eyes injures Blood (Heart); excessive lying down injures Qi (Lungs); excessive sitting injures muscles (Spleen); excessive standing injures bones (Kidneys); excessive exercise injures sinews (Liver). (Simple Questions Ch.23)

10. How does overwork eventually lead to Yin deficiency?

→ Daily work uses Post-Heaven Qi which replenishes quickly with rest. But if Qi cannot keep up with demands, the body draws on Yin substances (the 'savings account') to meet energy needs. Sustained overwork over years thus depletes Yin, especially Kidney-Yin. Short holidays cannot restore Yin — requires sustained lifestyle change.

11. What tongue changes indicate antibiotic use? Beta-blocker use?

→ Antibiotics: peeled tongue in patches (patches without coating) = Stomach-Yin deficiency. Beta-blockers: slow, deep pulse — pulse CANNOT be used for diagnosis in this patient.

12. Describe Wind-Heat vs Wind-Cold entry routes and key distinguishing signs.

→ Wind-Cold enters via skin → Taiyang channels. Wind-Heat enters via nose and mouth → Lung/LI channels. Wind-Cold: severe chills > fever, no thirst, thin white coating, superficial TENSE pulse. Wind-Heat: mild chills < fever, thirst, thin yellow coating, superficial RAPID pulse, possibly sore throat.

13. What is the Shaoyang stage and what are its characteristic symptoms?

→ The intermediate/half-exterior half-interior stage. Pathogen failed to go fully interior, anti-pathogenic Qi insufficient to push it back out. Classic: alternating chills and fever, fullness costal/hypochondriac, anorexia, vomiting, bitter taste, dry throat, blurred vision, string-taut pulse. Harmonize Shaoyang (Xiao Chai Hu Tang).

14. Name the three main Liver-related disharmonies arising from anger/emotional disturbance.

→ (1) Liver Qi Stagnation — anger bottled up, key: Wiry pulse, possible distending pain. (2) Liver Yang Rising — anger expressed, + Kidney Yin def; headache, dizziness, red face, tinnitus. (3) Liver Fire Blazing — intense/hot personality + Heat constitution; severe ascending symptoms, red tongue with yellow coat.

15. What are the 'counteracting emotions' and give two examples.

→ Each emotion counteracts another emotion via the Five Element Control Sequence: Sadness controls Anger (Metal controls Wood); Fear controls Joy (Water controls Fire); Anger controls Pensiveness (Wood controls Earth); Joy controls Worry (Fire controls Metal); Pensiveness controls Fear (Earth controls Water).

16. Describe Joy in its broad TCM sense (not ordinary happiness) and its effect on the Heart.

→ Joy in this sense means overstimulation, excessive excitement, or craving for constant stimulation — not simple happiness. It slows and scatters Heart-Qi, causing poor concentration, disturbed sleep, and over-talkativeness; long-term it can lead to Heart-Qi/Blood deficiency or, in a Hot constitution, Heart-Fire.

17. Describe the three-stage progression of a climatic invasion from Exterior to Interior, and the key sign marking each transition.

→ (1) Exterior: aversion to cold WITH fever, Floating pulse — release the Exterior. (2) Half-Exterior/Half-Interior (Shaoyang): alternating chills and fever, Wiry pulse — harmonize. (3) Interior: aversion to cold has resolved, Deep pulse — clear/warm/regulate the organ directly. Aversion to cold disappearing is the most reliable marker of the Exterior-to-Interior shift.

18. What signs result from excessive sexual activity depleting Kidney-Essence?

→ Sore weak lower back/knees, dizziness, tinnitus, premature greying, poor memory, and weak Will (Zhi) — the classic Kidney-Essence deficiency picture.

19. What classic presentation suggests a parasite-related pattern, especially in a child?

→ Periumbilical pain that comes and goes, poor appetite alternating with excessive hunger, teeth-grinding during sleep, pale patches on the face, and itchy nose/anus.

20. Why does old, unresolved physical trauma matter diagnostically even after the tissue has healed?

→ Trauma causes local Qi and Blood stagnation at the injury site that can persist long after visible healing — a classic hidden cause of chronic, fixed, localized pain that doesn't cleanly fit a Zang-Fu pattern. Always ask about injury history, including old surgical scars.

Section B • Multiple Choice

1. A patient has a pale tongue, clear copious urine, and loose stools. Which Eight Principle does this point to?

→ B. Cold — Pale tongue, clear urine, and loose stools are all classic Cold signs — the body's fluids are not being 'cooked' or consumed by Heat.

2. Which pulse quality is most associated with the Exterior pattern?

→ C. Floating — Floating reflects Qi rising to fight an invading exterior pathogen at the superficial level — the classic Exterior pulse.

3. Pain that is relieved by pressure is characteristic of which pattern?

→ B. Deficiency — Deficiency pain (Empty pain) is dull and alleviated by pressure/massage, unlike Excess pain which is aggravated by pressure.

4. True Cold with False Heat presents with a flushed face and thirst, but what gives away the TRUE cold underneath?
→ B. Patient wants to cover up despite the flush, prefers warm drinks — In true Cold/false Heat, surface signs mimic Heat (flushed face, feeling hot) but the patient still seeks warmth and covers — revealing the underlying Cold forcing Yang to the surface.
5. Which of the following is the Yin grouping in the Eight Principles?
→ B. Interior, Cold, Deficiency — Yin groups Interior, Cold, and Deficiency together; Yang groups Exterior, Heat, and Excess.
6. Which stage sits between Exterior and Interior, marked by alternating chills and fever?
→ B. Shaoyang — the pathogen is blocked from going fully interior, but the body can't fully expel it either; harmonize with formulas like Xiao Chai Hu Tang.
7. Excessive sexual activity is understood to primarily deplete which substance?
→ B. Kidney-Essence (Jing) — the substance governing reproduction, growth, and aging.
8. Periumbilical pain that comes and goes, plus teeth-grinding in sleep, is classically associated with:
→ B. Parasites — classic presentation includes intermittent periumbilical pain, appetite irregularity, teeth-grinding, and facial pale patches.
9. Which emotion can cause disease from sheer intensity alone, without needing to be long-lasting?
→ C. Shock — the one exception to the duration-or-intensity rule; a single instant of shock can cause disease.

Section C • True/False

1. Summer-Heat is the only one of the Six Excesses that can occur in any season.
→ FALSE — False — it's the opposite. Summer-Heat is the ONLY excess that can occur exclusively in summer; all the others can arise in any season.
2. Yang-Deficiency cold sensation is relieved by covering with blankets, while exterior Aversion to Cold is not.
→ TRUE — True. This is the key differentiator: Yang-Deficiency cold IS relieved by covering (chronic, no fever); exterior Aversion to Cold is NOT relieved by covering (acute, with simultaneous fever).
3. According to Maciocia, a brief but very intense emotional event can never become a cause of disease — only long-lasting emotions count.
→ FALSE — False. An emotion becomes a cause of disease if EITHER it is long-lasting (months/years) OR very intense even if brief — and Shock can cause disease almost instantly due to sheer intensity.

Section D • Case Study

1. Which Eight Principle pattern is this, and what TWO features confirm it is still Exterior?
→ Exterior pattern. Confirmed by: (1) simultaneous fever AND aversion to cold, and (2) a Floating pulse — both are exterior hallmarks.
2. Is this Wind-Cold or Wind-Heat? What features led you there?
→ Wind-Cold. Chills/aversion to cold predominate over the fever, there's no thirst mentioned, the coating is thin WHITE (not yellow), and the pulse is Floating+TIGHT (not Floating+Rapid) — Tight indicates Cold.
3. Why does he specifically want to be covered, and what would it mean if he didn't?
→ Wanting to be covered fits straightforward Cold — nothing unusual here. It would only become diagnostically interesting (True Cold/False Heat) if he showed a FLUSHED face yet still wanted to cover up — that combination would reveal interior Cold forcing Yang to the surface.

Pattern Reveal

- ★. Full pattern + reasoning
→ Wind-Cold invasion (Taiyang stage, Shang Han Lun framework). Reasoning: Exterior (sudden + floating pulse) + Cold (chills > fever, no thirst, thin white coat, Tight pulse). Treatment principle: release the Exterior with warm, acrid herbs (e.g. Gui Zhi Tang) — never tonify, or you trap the pathogen inward.



EAM310 • Worksheet Answer Key

Pulse Diagnosis & Tongue Diagnosis

Jon Centeno • VUIM DAChM

Answer Key — cover this page while testing yourself

✨ Pulse Diagnosis

Section A • Active Recall

1. What three qualities define a normal pulse?

→ Stomach-Qi (gentle, calm, slightly slow), Spirit (soft but with strength, regular), and Root (deep level + Rear position clearly felt). Together they reflect a good state of Qi, Mind, and Essence respectively.

2. Where are the Front, Middle, and Rear positions located, and what Burner does each correspond to?

→ Front (cun/inch) = Upper Burner (Heart-Lung); Middle (guan/gate) = Middle Burner (Spleen-Stomach/Liver); Rear (chi/foot) = Lower Burner (Kidney). Index finger=Front, middle finger=Middle, ring finger=Rear.

3. Floating + Tight pulse with a recent cold — what's the diagnosis?

→ Wind-Cold invasion (exterior Cold pattern).

4. Floating + Rapid pulse — what's the diagnosis?

→ Wind-Heat invasion (exterior Heat pattern).

5. What does a Deep + Weak pulse indicate? Deep + Full?

→ Deep + Weak = Qi/Yang deficiency. Deep + Full = stasis of Qi or Blood in the interior, or interior Cold/Heat.

6. Differentiate Knotted, Hasty, and Intermittent pulses.

→ Knotted = SLOW + irregular stops (Cold + Heart-Yang deficiency). Hasty = RAPID + irregular stops (extreme Heat, Heart-Qi deficiency, Heart-Fire). Intermittent = stops at REGULAR intervals, any rate (serious internal Yin-organ problem).

7. What are the four main indications of a Slippery pulse?

→ Phlegm, Dampness, retention of food, and pregnancy.

8. What does a Wiry pulse indicate (three conditions)?

→ Liver disharmony, pain, and Phlegm.

9. What is the difference between a Fine and a Minute pulse?

→ Fine = thinner than normal (Blood deficiency, or Dampness with severe Qi deficiency). Minute = even thinner/smaller than Fine, extremely hard to feel (severe deficiency of BOTH Qi and Blood).

10. A pulse that is Hollow — what does it indicate, and what does it feel like?

→ Felt superficial and deep but EMPTY in the middle (like a scallion stalk). Indicates hemorrhage; if also Rapid, indicates forthcoming blood loss.

11. What is a Moving pulse and what does it indicate?

→ Round like a bean, short, 'trembles,' no head/tail, somewhat slippery. Indicates shock, anxiety, fright, or extreme pain — often in people with deep emotional problems from fear.

12. Give the normal pulse rate range for an adult aged 16-35.

→ 76 beats per minute.

13. What does the Hammer & Bilton companion mean by a 'pseudo-arrhythmia'?

→ An irregularity that mimics pathology but stems from a benign cause — patient shifting position, talking, irregular breathing, or normal sinus arrhythmia — rather than a true Knotted/Hasty/Intermittent pulse.

14. What are the six ways to classify the 28 pulse qualities?

→ By Depth (Floating, Deep, Hidden, Firm, Leather), Rate (Slow, Rapid, Slowed-down, Hurried, Moving), Strength (Empty, Full, Weak, Scattered), Size (Overflowing, Fine, Minute), Length (Long, Short, Moving), Shape (Slippery, Choppy, Wiry, Tight, Moving, Hollow, Firm), and Rhythm (Knotted, Hasty, Intermittent).

15. Why do Small Intestine and Large Intestine have contradictory pulse position assignments across historical texts?

→ Reflects acupuncturist vs. herbalist perspective: acupuncturists assign LI/SI to the Front/Upper Burner position (matching their channel connection to Lung/Heart); herbalists assign them to the Rear/Lower Burner (matching where the organs physically sit).

16. What does a Tight pulse feel like, and what does it indicate?

→ Feels twisted, like a thick taut rope. Indicates Cold (exterior or interior) and pain. Tight + Floating = exterior Cold.

17. What is the difference between Overflowing and Full pulses?

→ Overflowing (Hong) is big, extends past its normal position, surges like a flood — indicates extreme/Full-Heat (or Empty-Heat if also Empty on pressure). Full (Shi) is full, hard, and rather long — a general Full-pattern pulse, not necessarily as dramatic in size as Overflowing.

18. A pulse described as 'a Wiry pulse felt only at the deep level' — name it.

→ Firm (Lao) pulse. Indicates Blood stasis, interior Cold (if also Slow), or pain.

19. What does a Leather pulse feel like and what severe condition does it indicate?

→ Hard/tight at the surface like a stretched drum, but completely Empty at the deep level. Indicates severe deficiency of Kidney-Essence or Kidney-Yin.

20. List the procedure steps for taking the pulse, in order, per FCM.

→ 1) Time (ideally early morning) 2) Level the arm 3) Place the fingers 4) Arrange the fingers (spacing by body size) 5) Regulate the fingers (initial overall read) 6) Move the fingers (lifting/pressing/searching/pushing/rolling) 7) Equalize your breathing (historic timing reference).

21. What does it mean for a pulse to have spirit but lack root?

→ It means Heart-Qi/Blood is in good shape (spirit intact — soft, regular, has strength) but the Kidneys are weak (root absent — deep level/Rear position can't be felt clearly). This is a classic sign of Kidney deficiency developing while the Heart is still compensating.

Section B • Multiple Choice

1. Which pulse quality is described as feeling like a scallion stalk — full at the surface and deep, but empty in the middle?

→ B. Hollow — Hollow (Kou) is the classic 'scallion stalk' pulse — felt at both superficial and deep levels but empty in between. Indicates hemorrhage.

2. A patient's pulse stops at IRREGULAR intervals and is RAPID. This pulse is called:

→ C. Hasty — Hasty = Rapid + irregular stops (extreme Heat, Heart-Qi deficiency, or Heart-Fire). Knotted is the Slow + irregular-stops version; Intermittent stops at REGULAR intervals.

3. Which TWO pulse qualities, when found together, most classically indicate Wind-Heat invasion?

→ C. Floating + Rapid — Floating (exterior) + Rapid (Heat) together indicate Wind-Heat. Floating + Tight would indicate Wind-Cold instead.

4. On the LEFT wrist, which organ is felt at the Middle (guan) position?

→ B. Liver — Left wrist: Front=Heart, Middle=Liver, Rear=Kidney(Yin). The Right wrist carries Lung/Spleen/Kidney(Yang).

5. A Slippery pulse found in an otherwise healthy young woman of reproductive age most likely indicates:

→ B. Pregnancy — Slippery is one of the four classic indications (Phlegm, Dampness, food retention, pregnancy) — in context of a healthy young woman with no other complaints, pregnancy is the most likely reading.

6. Per the Hammer & Bilton companion, what should you do FIRST when you feel an irregular pulse beat?

→ B. Re-check with the patient still and breathing calmly — Before committing to a true arrhythmia diagnosis, rule out a pseudo-arrhythmia by re-checking under calm, still conditions across multiple breath cycles.

Section C • True/False

1. A Knotted pulse is Rapid with irregular stops, while a Hasty pulse is Slow with irregular stops.

→ FALSE — False — reversed. Knotted = SLOW + irregular stops (Cold + Heart-Yang deficiency). Hasty = RAPID + irregular stops (Extreme Heat, Heart-Fire).

2. The Intermittent pulse, which stops at REGULAR intervals, is considered more serious than Knotted or Hasty.

→ TRUE — True. Intermittent (regular stops at any rate) points to a serious Yin-organ problem, and is generally regarded as the more serious of the three irregular-rhythm qualities.

3. A pseudo-arrhythmia should be diagnosed as a true Knotted/Hasty/Intermittent pulse without re-checking the patient.
→ FALSE — False. Always re-check with the patient still and breathing calmly across multiple cycles before committing to a true arrhythmia diagnosis — many irregularities are benign (movement, talking, breath-holding).

Section D • Case Study

1. Name the pulse quality being described.

→ Slippery (滑脉) — the classic 'pearls rolling in a dish' or 'beads under the finger' sensation.

2. What does this pulse quality indicate here, and what confirms it (rather than, say, pregnancy)?

→ Phlegm / Dampness. A Slippery pulse can also appear in pregnancy or food retention, but the swollen, greasy-coated tongue plus sticky taste and heaviness/fogginess point specifically to Phlegm-Dampness rather than a physiological cause.

3. Why does the patient feel 'heavy and foggy' specifically?

→ Dampness is heavy and turbid by nature and tends to obstruct the clear Yang from rising to the head — producing both physical heaviness and mental foggy/poor concentration.

Pattern Reveal

★. Full pattern + reasoning

→ Phlegm-Dampness (likely Spleen failing to transform fluids). Reasoning: Slippery pulse + greasy coat + swollen tongue + sticky taste is a textbook Phlegm-Damp cluster. Treatment principle: resolve Dampness and transform Phlegm while strengthening Spleen transportive function at the root.

✨ Tongue Diagnosis

Section A • Active Recall

1. A Red tongue with a thick coating vs. a Red tongue with no coating — what's the difference?

→ Red + coating = Full-Heat. Red + NO coating (peeled) = Empty-Heat (Yin deficiency).

2. What does a Purple tongue ALWAYS indicate, per Maciocia?

→ Blood stasis — NOT Qi stagnation, despite some Chinese texts saying otherwise. Reddish-Purple = stasis from Heat; Bluish-Purple = stasis from Cold.

3. Differentiate the 'Heart crack' from the 'Stomach crack.'

→ Heart crack = long, DEEP midline crack reaching the TIP (constitutional tendency to Heart patterns). Stomach crack = shallow, WIDE midline crack NOT reaching the tip (Stomach-Yin deficiency).

4. A Pale tongue that is wet vs. a Pale tongue that is dry — what's the difference?

→ Pale + wet = Yang deficiency (fluids not transformed). Pale + dry = Blood deficiency.

5. What does a Swollen tongue with a swollen RED tip indicate? Swollen red SIDES?

→ Swollen red tip = severe Heart-Fire. Swollen red sides = Liver-Fire.

6. What does Tooth-marking on the tongue indicate?

→ Spleen-Qi deficiency — the tongue is too swollen/large for the mouth and presses against the teeth.

7. What does a Deviated tongue (deviating to one side on protrusion) indicate?

→ Interior Wind.

8. Map the tongue microsystem: which organ corresponds to tip, center, root, and sides?

→ Tip = Heart/Lung; Center = Spleen/Stomach; Root = Kidney; Sides = Liver/Gall Bladder (front-sides = chest/Heart-Lung area).

9. What does a Stiff tongue indicate? A Flaccid tongue?

→ Stiff = interior Wind or Blood stasis. Flaccid = deficiency of Body Fluids.

10. Which Maciocia Tongue Companion chapters map to Week 4's content?

→ Ch.3 (Tongue Spirit and Body Color) and Ch.4 (Tongue Body Shape) — these mirror FCM Ch.23's body color/shape sections with much greater sub-variant detail.

11. What does a Long tongue indicate? A Short tongue?

→ Long = tendency to Heat, particularly Heart-Heat. Short = interior Cold (if Pale and wet) or extreme Yin deficiency (if Red and Peeled) — color/moisture determines which.

12. What does a Flaccid tongue indicate?

→ Deficiency of Body Fluids.

13. Differentiate a swollen RED tip from wide swollen PALE sides on the tongue.

→ Swollen red tip = severe Heart-Fire. Wide swollen pale sides = Spleen deficiency (the swelling reflects fluid retention from weak Spleen transformation, not Heat).

14. What does a swelling specifically in the front third of the tongue indicate?

→ Phlegm in the Lungs.

15. Orange/dark pale sides on the tongue indicate what severity of Liver-Blood deficiency?

→ A more SEVERE degree of Liver-Blood deficiency than plain pale sides.

16. What is the relationship between tongue body color and Blood/Nutritive Qi vs tongue coating and the Yang organs?

→ Tongue BODY (color/shape) reflects Blood, Nutritive Qi, and the Yin organs. Tongue COATING reflects the Yang organs, especially the Stomach, and the presence/nature of any pathogenic factor.

Section B • Multiple Choice

1. A tongue that is Red with NO coating at all (peeled, mirror-smooth) most strongly indicates:

→ B. Empty-Heat from Yin deficiency — Absence of coating with a red body signals Yin deficiency generating Empty-Heat — there's no active pathogen (which would produce a coating), just consumed Yin failing to control Yang/Heat.

2. A 'Stomach crack' on the tongue is best described as:

→ B. A shallow wide crack NOT reaching the tip — The Stomach crack is shallow and wide, stopping short of the tip — indicating Stomach-Yin deficiency. The Heart crack, by contrast, is long, deep, and reaches the tip.

3. Tooth-marking on the tongue edges is classically attributed to:

→ B. Spleen-Qi deficiency — Tooth marks occur because Spleen-Qi deficiency leads to fluid retention/swelling, making the tongue too large for the mouth and pressing against the teeth.

4. Per Maciocia, a Purple tongue body color ALWAYS indicates which underlying process?

→ B. Blood stasis — Maciocia is explicit that Purple always reflects Blood stasis, not Qi stagnation as some Chinese texts claim — Reddish-Purple from Heat, Bluish-Purple from Cold.

5. Which tongue zone corresponds to the Kidney in the standard tongue microsystem map?

→ C. Root — Root = Kidney. Tip = Heart/Lung, Center = Spleen/Stomach, Sides = Liver/Gall Bladder.

Section C • True/False

1. A Purple tongue, per Maciocia, always indicates Qi stagnation rather than Blood stasis.

→ FALSE — False. A Purple tongue ALWAYS indicates Blood stasis (not simply Qi stagnation) — Reddish-Purple traces back to a Heat origin, Bluish-Purple to a Cold origin.

2. Tooth-marking on the tongue edges indicates Spleen-Qi deficiency.

→ TRUE — True. A swollen tongue body pressing against the teeth, leaving tooth marks along the edges, is a classic sign of Spleen-Qi deficiency.

3. A coating that looks 'added on' rather than rooted in the tongue body indicates severe Stomach-Yin and Qi exhaustion.

→ FALSE — False. A coating without root (added-on look) indicates Stomach-Qi deficiency. COMPLETELY peeled/glossy coating is what indicates the more severe Stomach-Yin + Qi exhaustion.

Section D • Case Study

1. What does Purple on the tongue ALWAYS indicate, per Maciocia?

→ Blood stasis — always, regardless of location on the tongue.

2. Why specifically the SIDES, and what does that localize to?

→ The sides of the tongue map to the Liver in the tongue microsystem. Purple confined to the sides points to Blood stasis specifically involving the Liver — fitting with a gynecological Blood-stasis presentation, since the Liver stores Blood and governs the smooth flow of Qi and Blood through the Uterus.

3. What does the dark, clotted character of the menstrual blood add to the picture?

→ Dark, clotted blood is itself a hallmark of Blood stasis (along with fixed, stabbing pain) — it confirms and reinforces the tongue finding rather than introducing a separate pattern.

Pattern Reveal

★. Full pattern + reasoning

→ Liver Blood Stasis (gynecological). Reasoning: purple sides (Liver zone, always = stasis) + dark clotted menstrual blood + painful periods = converging evidence for Blood stasis centered in the Liver/Uterus. Treatment principle: invigorate Blood and resolve stasis, often combined with regulating Liver-Qi since Qi stagnation commonly underlies and perpetuates Blood stasis.



EAM310 • Worksheet Answer Key

Observation & Inquiry — 16 Questions

Jon Centeno • VUIM DAChM

Answer Key — cover this page while testing yourself

✨ Observation

Section A • Active Recall

1. What are the four observable signs of 'having Spirit' (Shen)?
→ Complexion has lustre; eyes have lustre/brightness; state of mind is alert and clear; breathing is even.
2. List the four attributes of a normal complexion.
→ Lustre, a subtle slightly reddish hue, 'contained'/veiled color, and moisture.
3. A patient's whole face is red vs. only their cheekbones are red — what's the difference?
→ Whole face red = Full-Heat. Red cheekbones only = Empty-Heat.
4. What does a green complexion with red eyes indicate? Grass-like green?
→ Green + red eyes = Liver-Fire. Grass-like green = collapse of Liver-Qi (serious).
5. Map the eye zones: which organ corresponds to corners, upper lid, lower lid, sclera, iris, pupil?
→ Corners = Heart; upper lid = Spleen; lower lid = Stomach; sclera = Lungs; iris = Liver; pupil = Kidney.
6. What does the tip of the nose reflect? The bridge of the nose?
→ Tip = Spleen. Bridge = Liver.
7. A tongue coating that looks 'added on' rather than growing from the tongue — what does this indicate?
→ A coating 'without root' — indicates Stomach-Qi deficiency.
8. Differentiate a partially peeled tongue from a completely peeled (glossy) tongue.
→ Partially peeled (geographic tongue) = Stomach-Yin deficiency. Completely peeled, mirror-smooth = severe Stomach-Yin exhaustion AND severe damage to Stomach-Qi.
9. What does a 'floating' complexion color indicate vs. a 'deep' complexion color?
→ Floating = exterior, mild, Yang condition. Deep = interior, severe, Yin condition.
10. What does a black complexion that is moist indicate vs. dry/burned-looking?
→ Black + moist = Cold. Black + dried-up/burned = Heat (usually Kidney-Yin Empty-Heat).
11. What does a 'distinct' complexion color indicate vs an 'obscure' one?
→ Distinct = Yang condition, Upright Qi is still intact. Obscure = Yin condition, Upright Qi is exhausted.
12. What does a thick coating on the tongue without root indicate, compared to a thin coating without root?
→ Both indicate the coating isn't truly rooted in the tongue body (Stomach-Qi deficiency), but a THICK rootless coating can also suggest a stronger pathogenic factor masking underlying deficiency — always integrate with other signs rather than reading coating thickness alone.
13. Whole eye red, painful, and swollen — what two patterns could this represent?
→ Invasion of exterior Wind-Heat OR rising of Liver-Fire.
14. What does swelling under the eyes indicate?
→ Kidney deficiency.
15. Describe the difference between Damp-Heat with Heat predominant vs Dampness predominant, as shown on the complexion.
→ Heat predominant: bright orange-yellow complexion. Dampness predominant: hazy, smoky, dull-yellow complexion.

Section B • Multiple Choice

1. A patient's complexion is bright white with a bluish tinge. This most likely indicates:
→ B. Yang deficiency with pronounced Cold — Bluish-white complexion specifically indicates Yang deficiency WITH pronounced Cold — more severe than plain bright-white (Yang deficiency alone) or dull-pale-white (Blood deficiency).

2. Which of these is NOT one of the four observable signs of 'having Spirit' (Shen)?

→ B. A strong, forceful pulse — The four signs are complexion lustre, eye lustre, clear/alert mind, and even breathing — pulse strength is a separate diagnostic category, not part of the Spirit observation itself.

3. The tip of the nose corresponds to which organ in facial zone mapping?

→ C. Spleen — Nose tip = Spleen; bridge of the nose = Liver.

4. A tongue coating that appears 'glued on' rather than emerging naturally from the tongue surface is described as lacking:

→ C. Root — A coating 'without root' looks artificially added rather than growing from the tongue body, and indicates Stomach-Qi deficiency.

Section C • True/False

1. A patient who 'has Spirit' (Shen) shows a lustrous complexion, vital eyes, a clear mind, and even breathing.

→ TRUE — True — these are exactly the four classic signs of 'having Spirit,' a key prognostic observation.

2. Cheekbone-only facial redness indicates Full-Heat, while redness across the whole face indicates Empty-Heat.

→ FALSE — False — reversed. Whole-face red = Full-Heat. Cheekbones only (malar flush) = Empty-Heat.

3. On the eye zone map, the pupil corresponds to the Kidney and the iris corresponds to the Liver.

→ TRUE — True. Pupil → Kidney, Iris → Liver, Sclera → Lungs, lids → Spleen/Stomach, corners → Heart.

Section D • Case Study

1. What does a sallow yellow complexion most commonly indicate?

→ Spleen deficiency / Blood deficiency — the Spleen is the source of Qi and Blood production (Post-Heaven root), so when it's weak, the complexion loses its normal lustrous quality and takes on a dull sallow-yellow cast.

2. How does the tooth-marked, pale tongue support this conclusion?

→ A pale tongue indicates Qi/Blood or Yang deficiency, and tooth-marking specifically indicates Spleen-Qi deficiency (the swollen tongue body presses against the teeth) — directly reinforcing the complexion finding.

3. Why does Spleen deficiency typically cause both fatigue AND poor appetite together, rather than just one?

→ The Spleen governs both transformation/transportation of food (appetite/digestion) and is a root source of Qi production (energy) — so when it's deficient, both functions fail together rather than in isolation.

Pattern Reveal

★. Full pattern + reasoning

→ Spleen-Qi (and likely Blood) Deficiency. Reasoning: sallow-yellow complexion + pale tooth-marked tongue + fatigue + poor appetite all converge on the Spleen as the primary deficient organ. Treatment principle: tonify Spleen-Qi (e.g. Si Jun Zi Tang as a base formula) to restore both digestive function and downstream Qi/Blood production.

🌟 Inquiry — 16 Questions

Section A • Active Recall

1. Why did Maciocia move 'Pain' to question #1 in his revised 16 questions, instead of 'chills and fever' (the classical first question)?

→ Because the classical order was biased toward acute exterior febrile disease (Shang Han Lun era China). Western patients most commonly present with pain as their chief complaint, so it makes more practical sense to ask about it first.

2. What are the three NEW questions Maciocia added to the classical 10?

→ Emotional state, sexual symptoms, and energy levels.

3. Desire for large amounts of cold water vs. thirst with small sips — what's the difference?

→ Large amounts of cold water = Full-Heat (any organ). Small sips = Yin deficiency (usually Stomach or Kidney).

4. Thirst but NO desire to actually drink — what does this indicate?

→ Damp-Heat — the Heat causes thirst, but the Dampness makes the person reluctant to actually drink.

5. Daytime sweating vs. night-time sweating — what's the difference?

→ Daytime sweating = Yang deficiency. Night-time sweating = Yin deficiency (occasionally Damp-Heat).

6. What does 'five-palm sweat' indicate?

→ Sweating on palms, soles, and chest — indicates Yin deficiency.

7. Pain before urination vs. during vs. after — differentiate.

→ Before = Qi stagnation in Lower Burner. During = Damp-Heat in Bladder. After = Qi deficiency.

8. Borborygmi with loose stools vs. borborygmi with distension and NO loose stools — differentiate.

→ With loose stools = Spleen deficiency. With distension, no loose stools = Liver-Qi stagnation.

9. What is the significance of profuse, oily forehead sweat that doesn't flow, 'like pearls'?

→ Collapse of Yang — a sign of imminent danger/death.

10. What is the difference between sweating only on the hands versus the whole body?

→ Sweating on hands only = Lung-Qi or Heart-Qi deficiency. Sweating on the whole body = Lung-Qi deficiency.

11. Frequent + copious urine vs frequent + scanty urine — differentiate.

→ Frequent + copious = Kidney-Yang deficiency. Frequent + scanty = Qi deficiency.

12. What does scanty, dark urine indicate?

→ Kidney-Yin deficiency.

13. What is the order of Maciocia's procedure for a full intake, from first contact to determining cause?

→ 1) Let patient speak freely 2) Ask specific questions to determine pattern + onset 3) Ask questions to exclude/confirm other patterns 4) Broadly follow the 16 questions 5) Ask about past diseases/operations 6) Look at tongue, feel pulse 7) Ask about family illnesses 8) Inquire about emotional/family/work life to find the cause.

14. Black or very dark stools indicate what pattern?

→ Blood stasis.

Section B • Multiple Choice

1. A patient reports thirst but states they have no real desire to actually drink water when offered. This combination most specifically points to:

→ C. Damp-Heat — Damp-Heat causes thirst (from the Heat) but reluctance to actually drink (from the Dampness) — a classic combined-pathogen presentation distinct from pure Heat or pure Yin deficiency.

2. Why did Maciocia move 'chills and fever' from question #1 (classical order) to question #12 in his revised 16 questions?

→ B. Western patients rarely present with acute febrile disease as their chief complaint — The classical 10-question order was biased toward acute exterior febrile disease common in historical Chinese clinical settings; Western patients more often present with pain or chronic complaints first.

3. Borborygmi (intestinal gurgling) WITH loose stools points toward:

→ B. Spleen deficiency — Borborygmi with loose stools indicates Spleen deficiency (failure to transform/transport); borborygmi with distension but NO loose stools instead points to Liver-Qi stagnation.

4. Pain that occurs specifically DURING urination points to:

→ B. Damp-Heat in the Bladder — Timing of urinary pain is diagnostic: before=Qi stagnation, during=Damp-Heat in Bladder, after=Qi deficiency.

Section C • True/False

1. Maciocia moved Pain to question #1 in his 16-question system because chills/fever is rarely a Western patient's chief complaint.

→ TRUE — True. The classical 10 questions (Shang Han Lun-era) asked Chills/Fever first for acute febrile disease; Maciocia reordered for Western practice, where Pain is the most common presenting complaint.

2. Thirst with a desire for only small sips of water typically indicates Full-Heat.

→ FALSE — False. Large amounts of cold water = Full-Heat. Small sips = Yin deficiency. No desire to drink at all despite thirst = Damp-Heat.

3. Pain that occurs BEFORE urination indicates Qi deficiency.

→ FALSE — False. Pain before urination = Qi stagnation. Pain during = Damp-Heat. Pain after = Qi deficiency.

Section D • Case Study

1. What does 'thirst without desire to drink' point toward, and why?

→ Damp-Heat. Heat creates the sensation of thirst, but the accompanying Dampness is turbid and obstructs normal fluid distribution/the sensation of needing fluid — so the patient feels thirsty in principle but has no real urge to actually drink.

2. How does the sticky taste + epigastric fullness support Damp-Heat specifically (vs simple Heat alone)?

→ A sticky taste and sensation of fullness/heaviness in the epigastrium are classic Dampness signs (Dampness obstructing the Middle Burner) — without these, you'd suspect plain Heat; with them, the picture becomes specifically Damp-Heat.

3. If this patient instead desired LARGE amounts of cold water, what would that change?

→ That would point to Full-Heat instead (without the Damp component) — large cold-water cravings indicate uncomplicated excess Heat consuming fluids, distinct from the Damp-obstructed thirst pattern described here.

Pattern Reveal

★. Full pattern + reasoning

→ Damp-Heat (likely Spleen/Stomach or Lower Burner depending on full presentation). Reasoning: thirst-without-desire-to-drink is one of the single most specific inquiry findings for Damp-Heat — confirmed here by sticky taste and epigastric fullness. Treatment principle: clear Heat AND resolve Dampness simultaneously — treating only one half leaves the other untouched.



EAM310 • Worksheet Answer Key

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Inquiry + Palpation & Palpation + Listening/Smelling

Answer Key — cover this page while testing yourself

✨ Inquiry + Palpation

Section A • Active Recall

1. What is the single general rule distinguishing Empty pain from Full pain?
→ Full pain is sharp/intense; Empty pain is dull/lingering. Full pain is aggravated by pressure; Empty pain is alleviated by pressure.
2. Differentiate pain from Qi stagnation vs. pain from Blood stasis.
→ Qi stagnation: distension MORE than pain, no fixed location, comes and goes with emotion. Blood stasis: severe, boring or stabbing pain, FIXED location.
3. A headache at the vertex — which channel, and most likely cause?
→ Terminal Yin channel — usually from deficiency of Liver-Blood.
4. A headache that is distending and throbbing — what's the likely cause?
→ Liver-Yang rising.
5. Epigastric pain that is dull and alleviated by eating vs. severe and aggravated by eating — differentiate.
→ Dull + alleviated by eating = Empty type (Stomach-Qi deficiency or Empty-Cold). Severe + aggravated by eating = Full type (food retention, Stomach-Heat, Damp-Heat, Fire, Blood stasis, or Liver-Qi invading Stomach).
6. Numbness of fingers/elbow/arm on ONE side only — what is the clinical concern?
→ Internal Wind and Phlegm — this may indicate the possibility of an impending Wind-stroke (Western: stroke warning sign).
7. On abdominal palpation, what does a mass that MOVES under the fingers indicate vs. a FIXED hard mass?
→ Moving mass = Qi stagnation. Fixed, hard, immovable mass = Blood stasis.
8. What does tenderness at Ren-17 (Shanzhong) on light palpation indicate?
→ Heart-Blood stasis.
9. What does it mean if chest palpation RELIEVES discomfort?
→ Deficiency condition of the Heart or Lungs (as opposed to tenderness on palpation, which indicates a Full condition).
10. What does pain from Cold feel like, and what aggravates/alleviates it?
→ Cramping, spastic pain. Aggravated by cold weather and cold foods/liquids; alleviated by application of heat. Common locations: epigastrium, abdomen, Uterus, joints.
11. What does pain from Damp-Heat feel like?
→ Burning pain accompanied by a feeling of fullness and heaviness. Common locations: forehead, epigastrium, abdomen, Uterus, Bladder, joints.
12. Differentiate a headache that's worse in the daytime vs the evening vs at night.
→ Daytime = Qi or Yang deficiency. Evening = Blood or Yin deficiency. Night-time = Blood stasis.
13. What does pain at the hypogastrium (lowest abdominal area) typically indicate?
→ Damp-Heat in the Bladder, or Liver-Fire infusing down to the Bladder.
14. What does the area below the xiphoid process ('Xu Li') reflect, and what does a hard/knotted finding there mean?
→ Reflects Upper Burner Qi (Lung-Qi, Heart-Qi, Gathering/Zong Qi). Hard and knotted = stagnation of Lung/Heart-Qi from bottled-up emotional tension. Too soft = Heart deficiency.

Section B • Multiple Choice

1. Pain that comes and goes with emotional state, causing more distension than actual pain, with no fixed location, is most characteristic of:
→ B. Qi stagnation — Qi stagnation pain is distension-dominant, fluctuates with mood, and has no fixed site — distinct from Blood stasis, which is severe, fixed, and stabbing.

2. On chest palpation, tenderness elicited even with LIGHT pressure at Ren-17 (Shanzhong) most specifically suggests:
→ B. Heart-Blood stasis — Tenderness at the center of the chest (Ren-17) on light palpation is a classic sign of Heart-Blood stasis.
3. Numbness localized to the fingers/arm on ONE side only is a clinical warning sign for:
→ B. Possible impending Wind-stroke — Unilateral numbness (especially first three fingers, arm, elbow) suggests internal Wind + Phlegm and may be an early warning sign of an impending stroke — a significant clinical red flag.
4. An abdominal mass that is FIXED, hard, and does not move under palpation most likely represents:
→ B. Blood stasis — Fixed, hard, immovable masses indicate Blood stasis; masses that move or come and go under the fingers instead suggest Qi stagnation.

Section C • True/False

1. An abdominal mass that MOVES under palpation suggests Blood stasis, while a FIXED hard mass suggests Qi stagnation.
→ FALSE — False — reversed. Masses that MOVE = Qi stagnation. Masses that are FIXED and hard = Blood stasis.
2. Tenderness at Ren-17 elicited even with light pressure is a classic sign of Heart-Blood stasis.
→ TRUE — True. Light-pressure tenderness at the chest center (Ren-17/Shanzhong) is a textbook sign of Heart-Blood stasis.
3. Unilateral numbness of the fingers and arm should be dismissed as a minor, clinically insignificant finding.
→ FALSE — False — this is a clinical red flag for possible impending Wind-stroke (internal Wind + Phlegm) and should never be dismissed.

Section D • Case Study

1. Is this Full or Empty type pain? What's the single biggest clue?
→ Full type. The pain is AGGRAVATED by pressure — Empty-type pain is, by contrast, relieved/alleviated by pressure. Severity and the sudden/aggravated character also point to Full.
2. What is the likely underlying cause, given foul breath + constipation + thick yellow coat?
→ Food stagnation transforming into Stomach-Heat (or straightforward Stomach-Heat/Fire from accumulated Excess). Foul breath and constipation reflect accumulated, putrefying material in the digestive tract generating Heat; the thick yellow coat confirms an active Heat/Excess process in the Stomach.
3. Why does eating specifically worsen this patient's pain?
→ Because the pathology is Excess accumulation in the Stomach itself — adding more food directly adds to the existing stagnation/Excess, aggravating the local obstruction and pain, unlike Deficiency pain which is often relieved by food/warmth.

Pattern Reveal

- ★. Full pattern + reasoning
→ Stomach-Heat from Food Stagnation (Full, Excess pattern). Reasoning: pressure-aggravated severe pain (Full) + foul breath/constipation/thick yellow coat (Heat + accumulation) + worse with eating (Excess feeding Excess). Treatment principle: clear Stomach-Heat and reduce/move the accumulation (purgative + heat-clearing approach) — tonifying here would worsen the stagnation.

✨ Palpation + Listening/Smelling

Section A • Active Recall

1. What is the single general rule for diagnosis by listening (hearing)?
→ A loud sound indicates a Full pattern; a weak sound indicates an Empty pattern.
2. What is the single general rule for diagnosis by smelling?
→ A strong, foul smell indicates Heat; absence of smell indicates Cold.
3. Sudden loss of voice vs. gradual loss of voice — what's the difference?
→ Sudden = invasion of exterior Wind-Heat. Gradual = deficiency of Lung-Qi or Lung-Yin.
4. Match the Five-Element voice types to their organs: shouting, laughing, singing, weeping, groaning.
→ Shouting = Liver (Wood). Laughing = Heart (Fire). Singing = Spleen (Earth). Weeping = Lung (Metal). Groaning = Kidney (Water).

5. What does a dry cough specifically indicate?

→ Lung-Yin deficiency.

6. Why is it diagnostically meaningful if a Wood-type person's body odor is scorched rather than rancid?

→ Rancid is the Wood type's CONSTITUTIONAL odor (not pathological on its own). A scorched smell (Fire's odor) contradicts their constitution and indicates a Heart pattern is present — this mismatch is actually a WORSE sign than an exaggerated rancid smell.

7. What is a 'pseudo-arrhythmia' and how do you clinically rule it out?

→ An irregularity that mimics a true arrhythmia (Knotted/Hasty/Intermittent) but stems from a benign cause like position change, talking, or breath-holding. Rule it out by re-checking the pulse with the patient still and breathing calmly across multiple cycles before committing to a diagnosis.

8. What does it mean if the apical (heart apex) pulse and the radial pulse disagree?

→ Indicates a poor prognosis.

9. Intestinal gas with no smell at all — what does this indicate?

→ Spleen-Qi deficiency.

10. What does it mean if breath odor is foul AND pungent, versus simply sour?

→ Foul + pungent = Damp-Heat in Stomach/Spleen. Sour = food retention, or Accumulation Disorder in children.

11. What does it mean if vomiting occurs with a loud noise vs a low quiet sound?

→ Loud noise = Full pattern, often with Heat (frequently Stomach-Heat). Quiet/low sound = Empty pattern (frequently Stomach deficiency and Cold).

12. What does a feeble apical (heart apex) beat without strength indicate?

→ Deficiency of Gathering Qi (Zong Qi).

13. What does cold skin specifically over the lower back, loins, and feet indicate?

→ Kidney-Yang deficiency.

14. Describe how a constitutional vs pathological body odor mismatch is diagnostically meaningful, using the Wood-type example.

→ A Wood-type person's natural constitutional odor is rancid — not pathological on its own. If that same person instead develops a SCORCHED smell (Fire's odor), it signals a Heart pattern has developed, contradicting their Wood constitution — and this mismatch is actually a WORSE sign than an exaggerated rancid smell would be.

Section B • Multiple Choice

1. A patient speaks very little and seems reluctant to talk. This voice pattern most often indicates:

→ B. Cold pattern or Lung-Qi deficiency — Reluctance to talk suggests either a Cold pattern or Lung-Qi deficiency — low energy/Qi to project the voice. Excessive talking, by contrast, suggests Heat.

2. Per the Five-Element voice correspondences, a 'groaning' voice quality points to which organ?

→ D. Kidney — Groaning corresponds to Kidney (Water element). Shouting=Liver, Laughing=Heart, Singing=Spleen, Weeping=Lung.

3. Per Hammer & Bilton, what is a 'pseudo-arrhythmia'?

→ B. An irregularity caused by a benign, transient factor rather than true pathology — A pseudo-arrhythmia mimics a true Knotted/Hasty/Intermittent pulse but is actually caused by something benign — patient movement, talking, breath-holding, or normal sinus arrhythmia.

4. A discrepancy between the apical (heart apex) pulse and the radial wrist pulse generally indicates:

→ C. Poor prognosis — When the apical pulse and radial pulse disagree, this is considered a poor prognostic sign and warrants closer clinical attention.

Section C • True/False

1. A loud, forceful sound on auscultation generally indicates a Full pattern, while a weak sound indicates an Empty pattern.

→ TRUE — True — this is the single general rule underlying all of listening diagnosis in FCM Ch.26.

2. An absent or faint body odor generally points toward Heat, while a strong foul odor points toward Cold.

→ FALSE — False — reversed. STRONG/FOUL smell = Heat. ABSENCE of smell = Cold.

3. A discrepancy between the apical (heart apex) pulse and the radial wrist pulse generally indicates a good prognosis.

→ FALSE — False — it indicates a POOR prognosis and warrants closer clinical attention.

Section D • Case Study

1. Which organ does this voice/odor pattern correspond to in Five-Element correspondence?

→ Liver (Wood). Shouting = Liver's voice, Rancid = Liver's odor — both point to the same organ independently, then converge.

2. Does the LOUD quality of the voice tell you Full or Empty? Why?

→ Full. The single general rule for listening diagnosis is: loud sound = Full pattern, weak sound = Empty pattern — a loud, shouting-toned voice indicates an active, Excess-type process.

3. How does the Wiry pulse complete (not just add to) this picture?

→ Wiry is THE signature Liver-disharmony pulse quality — its presence alongside a Liver-typed voice and Liver-typed odor means all three independent diagnostic methods (listening, smelling, pulse) agree on the same organ, which is exactly the kind of convergence that makes a diagnosis solid rather than speculative.

Pattern Reveal

★. Full pattern + reasoning

→ Liver-Qi Stagnation tending toward Liver-Fire (chronic frustration as the causative emotion). Reasoning: shouting voice + rancid odor + Wiry pulse + chronic frustration/irritability all converge independently on the Liver, with the loud/Full quality suggesting an active Excess process rather than simple deficiency. Treatment principle: soothe Liver-Qi and, if Heat signs are also present (red sides, yellow coat), add Liver-Fire-clearing herbs.